



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : محاسن محمد يوسف احمد

المحافظة : الجيزة الحي / المدينة : بولاق الدكرور العنوان : ..

رقم البطاقة : 26005062101121 رقم التليفون : 01143438108 تاريخ تسجيل الإستمارة : 20-09-2026

العمر (تاريخ الميلاد) : 06-05-1960 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي بروتوكول علاج كيماوي خط ثاني للورم المتنشر (Platinum +Gemcitabine) [14245]

قسم العلاج :
الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: محاسن محمد يوسف احمد National ID: 26005062101121	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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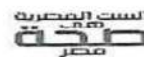
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Platinum plus gemcitabine chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



06/11/2026

MDT FORM

Name:	Mahassen Mohamed Yousef Ahmed	MRN:	2614
Age:	66 Y	ID:	26005062101121
Menopausal Sta	postm	Phone:	01092135718
NRS SCORE:	2	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

R

L

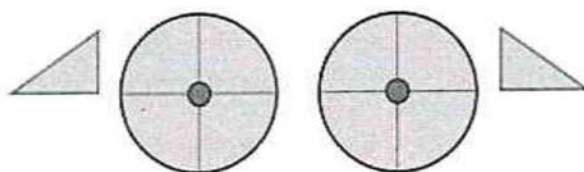
BX:
ER: PR: HER2: KI67:

Sonomammography:Mets Workup:History:

postm rt breast cancer met to liver lung bone ct4d n1 m1 ldc 2 er8/8 pr6/8
her2sish amplified initial pet ct bilat axillary med ln cervical 2 hfl seg 8 ,5
largest 1.2 cm rec taxol herceptin st arimidex from 2/2025 cts 1/2025 non
contrast no hfl cts 3/5/2026 comp to 1/2025 non contrast newly developed
hfl seg 8 m 6x5 cm .2 smaller lesions seg 7 m 1.5x1.3 rd of pul nodules pd of
vertebral mets for mdt

Recommendations:MDT Outcome & Plan

After reviewing CTs, the only progressive/new Site of mets is the hypodense
HFL 6 cm and two other small 1-2 cm.
Bony lesion is tiny and not present on Bone scan.
All other metastatic sites are SD or RD.
--> For triphasic CT & possibility of doing local ablation to liver lesions with
intervention Radiology and continue on AI + Herceptin.
If not possible or patient is clinically progressing : Shift to Xeloda -Tykerb



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



٢٠٢٠/١٢

٢٦٠٠٥٠٦٢١٠١١٢١

ربة منزل



متزوجة

مقيمة

أنثى

محمد احمد على براهيم

البطاقة سارية حتى ٢٠٢٧/١٢/٢٤



جمهورية مصر العربية
بطاقة تحقيق الشخصية

محاسن
محمد يوسف احمد

٢٣ ش سيد طه السمكري - مبانى كفر طاهر مس
بولاق الدكرور - الجيزة

٢٦٠٠٥٠٦٢١٠١١٢١

IR8050314

ECHO REPORT

NAME: *aswani*
ID NUMBER: *1*

DATE: 8/2/2026
SEX: ♀ AGE: 63

EF	68%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.3	2-4cm	LA	3.6	2-4cm
FS	29%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *Normal*
LV Wall Thickness *Normal*
LV Diastolic Function *GI*
SWMA *No Ruma*
LA and Ao Root Dimensions *Normal*
Mitral Valve *Normal*
Aortic Valve *Normal*
Tricuspid Valve *Normal*
Pulmonary Valve *Normal*
Right Side and Systolic Function
Intracardiac Masses and Effusion

TAPSE = 2.3

DR. *[Signature]*



SCAN 8 LABS

Name: Mahasin Mohammed Youssef Ahmed

Branch: Dokki

Ref. Physician Prof: Goustav Roussey Egypt

Date: 05-03-2026 09:29

Medical No.: 327759

مركز النيل

ISOTOPIC BONE SCAN REPORT

- **Indication:** 66-year-old female with history of breast carcinoma. Evaluation for possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2-3 hours post intravenous injection.

FINDINGS:

Compared to the prior bone scan dated 25/06/2025, the current scan shows:

- Stationary course of the old osseous lesions at the sternum, D1, D7 vertebrae.
- Regression of the previously documented osseous lesions at the right iliac bone and right 2nd rib.
- No new lesions could be depicted.
- Degenerative/Arthritic changes are seen at different levels of the spine, sternum, both shoulders, as well as the weight-bearing joints.

IMPRESSION:

- Mild regressive course of the metastatic bone disease as above detailed, with no new lesions depicted.
- Follow up is recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Name : Mohasin Mohammed Youssef Ahmed

Date : 25-06-2025 06:15

Branch : Maadi

Medical No. : 136567

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 65-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

Compared to last bone scan dated 27/08/2024:

- Rather stationary course of the previously noted osseous lesions at right 2nd rib, DV1 and DV7.
- Newly developed osseous lesion is seen at the right iliac bone.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Metastatic osseous lesions at the aforementioned sites with newly developed deposit at the right iliac bone, for follow up to monitor therapy response.

*Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine*

Patient Name: Mahasen Mhmdiosf Ahmed	Date : 07/07/2025
Exam : C.T CHEST	Age : 65 Year(s)
Medical ID: 2700/2024 690	Pat. ID: 94710

***NON-CONTRAST CT SCAN OF THE CHEST ,
ABDOMEN AND PELVIS : -***

Findings:

- **Clear both lung fields. ,**
- Small subcarinal enlarged L. N measures 6 x3 mm likely reactive.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- **Average size liver showing homogenous density. No gross focal lesions**
- **Average size spleen , no focal lesions.**
- ***Average size gall bladder , no stones . .**
- *No dilated intra or extra-hepatic bile ducts .
- *Urinary bladder shows no masses or stones.
- *Normal C.T. appearance of the pancreas , adrenal glands, both kidneys ,bowel loops & other pelvic organs.
- *No retroperitoneal lymphadenopathy .
- *No ascites .
- **The examined bones show no worrisome lytic or sclerotic lesions.**

Opinion:

- **No available clinical data**
- **Unremarkable study.**

• **DR/Reham khedr...MD**

Signature Dr.Mohamed Hassan

09/07/2025 7:21 PM

Signature Dr.Mohamed Hassan

09/07/2025 7:21 PM

Name : Mohasin Mohammed Youssef Ahmed

Date : 25-06-2025 06:15

Branch : Maadi

Medical No. : 136567

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 65-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- Abnormal tracer uptake is noted at right 2nd rib, DV1, DV7 as well as right iliac bone.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Metastatic osseous lesions at the aforementioned sites, for follow up to monitor therapy response.

*Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine*



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : مخبر محمد يوسف احمد

Sex:

3- Medical number:

Female

B3225738

Old Med i

DN2 Med i
2700-2024

2-ID number:

Patient Age : 84 Y



Family history	Family History -ve	Phone	Phone 01143438108	BHT	DN2	REASON OF MDT	Reason of MDT Adjuvant
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT 15/10/2024
Co morbidity	Co morbidity HTN & knee replacement b. Ubla c. chronic tool kidney	Menopausal status	Menopausal status POSTMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging : RTBREAST ELEPISILA TUR AXILY LN BIRADS 5			Mets Workup : CT EOUT AN TRA5R RT axially L.N prominent supra clavicular Fissure pulmonary nodules few mediastinal D1 small lytic lesion		
BX IDC BX COMMENTS	ER 8/8 BX COMMENTS	PR 5/8 BX COMMENTS	Her 2 Neu Her 2 Neu COMMENTS +ve	SISH SISH COMMENTS	Ki67
T 4b		N 1		M 1 bone, nodes, lung, Liver	

Sonomammography

RT LOCALLY ADVANCED BREAST CANCER WITH N1 DISEASE
THE REPORT CLAIMED LEFT MICROCALCIFICATION ... ON REVIEWING THE FILMS NOTHING DETECTED

PATIENT HISTORY

last mdt:NACT 6 CYCLE TCHP THEN RT MRM
PET-CT done and confirmed the presence of multiple metastasis

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for weekly taxol-IV trastuzumab (stop carboplatin as no need to give combination chemo)
zometa
palliative radiotherapy

SIGNATURE			
MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	



Patient Name: Mahasen Mhmdiosf Ahmed	Date : 29/01/2025
Exam : C.T CHEST	Age : 64 Year(s)
Medical ID: 2700/2024	Pat. ID: 94710

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Compared to previous study done outside dated 07/08/2024 with no available bone window for optimal comparison.

Findings:

I Chest:-

➤ Stationary course regarding:

- Right breast retroareolar lesion measuring 5 x 3.3 cm still seen reaching the nipple with overlying diffuse skin thickening.
- Bilateral prominent supraclavicular lymph nodes largest on left measuring 8 x 7 mm.

➤ Regressive course regarding:

- Multiple right axillary prominent lymph nodes being millimetric in today's study.
- Bilateral pulmonary and fissure based nodule largest at right upper lobe posterior segment measuring 7 mm compared to 14 mm previously.
- Few right para tracheal and retrocaval lymph nodes being millimetric in today's study.

➤ Currently noted middle lobe reticulonodular densities with tree in bud nodularities.

- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

Patient Name	: الأستاذة / محاسن محمد يوسف	Request No.	: 13002383
Patient No.	: 26005062101121	Request Date	: 17/08/2024
Path No.	: SR-168/24	Verification Date	: 28/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 64 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:

-1 block labeled by (4198/24) and 3 slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
 - * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
 - * DCIS: Focal.
 - * Angiolinvasion: No.
 - * Perineural invasion: No.
 - * IPT:
 - ER: Strong nuclear reaction in 80% of tumor cells (score 8 /8)
 - PR: Strong nuclear reaction in 25% of tumor cells(score 6 /8).
 - Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
 - SISH: Amplified with clusters.
- Average Her2 copy number: >6.
Her2/CEP 17 ratio: >4 .

Diagnosis:

Right breast cancer, Slide revision:
Invasive duct carcinoma GII with focal DCIS.
IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 6 /8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with clusters.

Verified by :

DR.Menna Nabil



Patient Name	: الأستاذة / محاسن محمد يوسف	Request No.	: 13002383
Patient No.	: 26005062101121	Request Date	: 17/08/2024
Path No.	: SR-1168/24	Verification Date	: 28/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 64 Y	Department	: Out Patient
Referring Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil





NAME	Mahasen mohammad Youssef Ahmed	SEX	Male
NO.	Q42448	DATE	9/28/2024 2:16:20 PM

Findings:

+ Right breast:

- Diffuse increased FDG uptake corresponding to **multiple irregular breast lesions**, example is retro-areolar lesion tethered to the skin with SUVmax 13.9 over 5.4x 2 cm.
- Nodular infiltration of the chest wall muscles, SUVmax 9.7 over 1.6 cm lesion within the pectoralis major muscle.

+ Left breast:

- Mild FDG uptake within lower outer quadrant nodule with SUVmax 1.9 and measures 1.1 cm.
- 6 o'clock position nodule with no FDG uptake.

+ Lymph nodes: increased FDG uptake corresponding to discrete and amalgamated lymph nodes:

- Bilateral axillary levels I, II and right internal mammary nodes, SUVmax 7.6 over 1.2 cm level II node, SUVmax 2.2 over 0.9 cm right internal mammary node, SUVmax 4.4 over 0.7 cm left axillary node.
- Cervical including bilateral levels II, II and IV, SUVmax 4.2 over 1 cm left supraclavicular node.
- Mediastinal including prevascular, para-tracheal, subcarinal, hilar and posterior mediastinal, SUVmax 14.1 over 1.9 cm right hilar node.



Mahasen mohammad Youssef Ahmed

Male

Q42448

9/28/2024 2:16:20 PM

Whole body ^{18}F -FDG PET/CT

Clinical History

↓ A patient with history of right breast cancer.

Procedure:

- Whole body ^{18}F -FDG images from skull base to mid thighs. CT images of the same level.

Injected dose: 321 MBq

BGL: 99 mg/dl

IV contrast: No

Injection time: 12:13

Weight: 98 Kg

Oral contrast: water

Acquisition time: 01:21

Liver SUVmax: 3.3

Time/bed position: 2min

Opinion

- ✓ Metabolically active right breast neoplasm with metastatic lymph nodal, pulmonary, hepatic and osseous lesions.



NAME	Mahasen mohammad Youssef Ahmed	SEX	Male
NO.	Q42448	DATE	9/28/2024 2:16:20 PM

↓ Lungs:

- Increased FDG uptake corresponding to bilateral pulmonary nodules, SUVmax 1.6 over 1 cm middle lobe nodule, SUVmax 1.8 over 0.6 cm left upper lobe nodule.

↓ Liver:

- Increased FDG uptake corresponding to two lesions involving segments VIII and V , SUVmax 10.1 over 1.2 cm segment VII lesion.

↓ Bone marrow,

- Increased FDG uptake corresponding to osseous lesions involving few right ribs, DV1, sacrum and pelvic bones, SUVmax 12.1 over right iliac bone and SUVmax 5.6 over right 2nd rib.

↓ Nodular thyroid gland demonstrating patchy FDG activity.

↓ The pancreas, adrenals, kidneys, aorta and IVC appear unremarkable.

Dr. Asmaa Magdy, MSc

Dr. Mohammed Omar, MD

جوستاف
روستاف
مستشفى
مستشفى



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
العربية
السعودية
KSA

09/16/2026

MDT FORM

Name:	Mahassen Mohamed Yousef Ahmed	MRN:	2614
Age:	66 Y	ID:	26005062101121
Menopausal Sta	PS ECOG 2	Phone:	01092135718
NRS SCORE:		Marital Status:	
BMT:		Family History:	- Ve
CO Morbidity:	HTN	Reason Of MDT:	DP

Clinical Data:

BX:

ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

History:
postm rt breast cancer met to liver lung bone ct4d n1 m1 idc 2 er8/8 pr6/8
her2sish amplified initial pet ct bilat axillary med in cervical 2 hfl seg 8 ,5
largest 1.2 cm rec taxol herceptin st arimidex from 2/2025 cts 1/2025 non
contrast no hfl cts 3/5/2026 comp to 1/2025 non contrast newly developed hfl
seg 8 m 6x5 cm .2 smaller lesions seg 7 m 1.5x1.3 rd of pul nodules pd of
vertebral mets
CT CAP (8-9-2026) : CT CAP 8-9*-2026 >> DP regarding pulmonary nodules
, HFLs , bony metastasis
newly developed multilevel lt axillary LNs largest 13*18 mm
newly developed small lesions LNs at D2 D12
labs >>> S. cr 2.7 urea 125 bil 10/9 ast 520 ALT610
PAUS >>> R

Recommendations:

MDT Outcome & Plan
In view of high burden disease, shift to Gemzar/Carboplatin/Tykerb with close
monitoring of response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء الاشقر	د. اميره عيسوي				

16/9/26
16/9/26

Patient's Name: Mohasin Mohammed Youssef Ahmed
Date: 2026-09-08
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTI-SLICE NON-CONTRAST CT SCAN OF THE CHEST AND ABDOMEN REVEALED:

A Known case of right breast cancer and right renal mass.

- Large Heterogenous right breast soft tissue masses. With axillary LNs, and skin thickening.
- Newly developed multilevel suspicious looking left axillary LNs, the largest (2.2x1.7cm)...For sono-mamography

Chest findings

- Bilateral multiple small pulmonary nodules seen scattered in lung parenchyma on both sides, the largest seen at the lingula measures about (10mm)....Metastatic
- Associated right upper lung lobe areas of ground glass opacities and diffuse interlobular septal thickening and peribroncho-vascular interstitial thickening.....? Neoplastic process....? Others for follow up.
- Clear pleural surfaces with no evidence of pleural effusion.
- Normal cardiac size, no pericardial effusion.
- Left supraclavicular LNs, the largest 13 x 18mm

Abdomen & Pelvis findings

- Enlarged liver. With multiple bi-lobar variable sized hypodense suspicious focal lesions the largest seen at segment VII of right liver lobe measuring about (7.7x4.8cm).
- No dilated intra-hepatic biliary radicles.
- Normal caliber patent Portal Vein
- Normal appearance of the pancreas.
- Normal size, shape and density of the spleen. No focal lesions.
- Normal size, shape and parenchymal thickness of the both kidneys. No detected stones or back pressure changes.
- Normal both supra-renal glands, no masses.
- No ascites
- Normal filling of the urinary bladder, no vesical stones or fungating masses.
- Newly developed small lesions at D2 and D 12 vertebrae with collapsed D 2 vertebral body.

CONCLUSION:

- Progressive course regarding the pulmonary, hepatic and bony metastatic deposits.
- Multilevel left axillary suspicious LNs as described ...for dedicated study.

Ibrahim